

BODY DECODE • FOUR EXPLANATIONS

Before You Accept It's Your Hormones

Four explanations for tired, wired and stuck. Only one of them ever gets checked.

Tired all the time. Awake at 3am. Body will not change.

You have almost certainly been given one explanation for that.

It is probably half right. Sixty per cent of the women who complete our profile are perimenopausal or past it, so hormones really are in the mix, and anyone telling you otherwise is selling something.

But here is what we watch happen. **The moment hormones get the blame, everything else stops being checked.** Nobody asks what your week actually costs you. Nobody asks whether you are eating enough for what you are doing. Nobody asks how long this has been going on, or whether it started before anything else changed.

You got an explanation, so the looking stopped.

This is not an argument that it is not hormonal. It is an argument that hormonal was the only thing anybody checked. There are four common explanations for the same three symptoms, most women have more than one running at once, and the plan is different for each.

1 One · Oestrogen is falling

The explanation you have already been given, and often a real part of it.

- ☐ **It used to sit on your hips and thighs, and over the last few years it has moved to your middle.**
- ☐ **Your cycle changed before anything else did.** Shorter, heavier, less predictable.
- ☐ **Sleep went first in the second half of the month,** before it went every night.
- ☐ **Joints ache in the morning** in a way they never used to.
- ☐ **It came on over years, not months.**

What changes if it is this one. The correction is not more effort, it is different timing. Training and food get organised around where you are in the month rather than against it. Strength work matters more than it used to, not less.

2 Two · The load is higher than the recovery

The one that gets mistaken for hormonal most often, because it produces the same 3am.

- ☐ **Your middle is where you have always carried it,** even at thirty.
- ☐ **You are wired at night and flat in the morning.**

- ☐ **Your arms and legs stayed lean while the middle filled out.**
- ☐ **You fall asleep fine, then wake around three and start thinking.**
- ☐ **It got worse in a period you could name.** A job, a loss, a move, a year.

What changes if it is this one. Nothing about the training gets harder. The work is in what surrounds it, because a body in protection will not spend what it does not think it can spare.

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Three · You are eating for half the life you are living

The one almost nobody self-identifies with, and the one that looks most like discipline.

- ☐ **You are the most careful person about food that you know.**
- ☐ **Your weight is stable and nothing changes anyway.**
- ☐ **You feel the cold more than you used to.**
- ☐ **Sessions that used to feel easy feel heavy,** and recovery takes days.
- ☐ **You eat least on the days you train hardest.**

What changes if it is this one. More, not less. Which is the hardest sentence in this guide to accept, because everything you have been told rewards the opposite.

A word of care. If food feels like something you are managing rather than eating, this guide is not the right tool and neither is another plan. Please talk to your GP.

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Four · Your breathing is interrupting your sleep

The one nobody screens for in women, because it does not look the way people expect.

Up to three quarters of women who have sleep apnoea have never been diagnosed. The screening was built around how it presents in men, so the picture people look for is snoring and daytime sleepiness. In women it more often shows up as fatigue that never lifts, waking in the early hours, low mood and morning headaches.

Which is the same list that gets called hormonal.

- ☐ **You wake with a headache more than once a week.**
- ☐ **You have been told you snore, or you wake with a dry mouth.**
- ☐ **You have woken gasping, or with your heart going.**

- ☐ The tiredness does not lift after a genuinely good night.
- ☐ It has been going on for years, and it predates any hormonal change.

This is the one we do not touch. We screen and refer, we do not diagnose. If you ticked two or more above, that is a conversation with your GP about a sleep study, and it is worth having before you spend another year on the other three. Nothing in this guide replaces that.

If you ticked boxes in more than one

Most women do. That is not the guide failing, it is the actual situation.

Two of these commonly run together, and the order matters. A body that is under-fuelled and under-slept will not respond to a plan built for falling oestrogen, however correct that plan is on paper. **The one that is furthest from normal gets addressed first**, not the one you have heard the most about.

Which is the whole reason we assess before we prescribe. Across the women we have profiled, only **18 in 100** came out able to absorb a hard training plan. The other eighty-two get handed one anyway.

Find out which one is yours.

The free 14-Day Body Decode Challenge scores five foundations on day one and works out which of the four is driving it, so the plan is built for the body you are actually in.



bodyrecode.au/challenge